



Clindamycin (systemic): Drug information

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For additional information see "[Clindamycin \(systemic\): Patient drug information](#)" and "[Clindamycin \(systemic\): Pediatric drug information](#)"

For abbreviations, symbols, and age group definitions [show table](#)

ALERT: US Boxed Warning

Colitis:



Clostridioides difficile-associated diarrhea (CDAD) has been reported with use of nearly all antibacterial agents, including clindamycin, and may range in severity from mild diarrhea to fatal colitis. Treatment with antibacterial agents alters the normal flora of the colon, leading to overgrowth of *C. difficile*.

Because clindamycin therapy has been associated with severe colitis, which may end fatally, reserve it for serious infections for which less toxic antimicrobial agents are inappropriate. Do not use clindamycin in patients with nonbacterial infections, such as most upper respiratory tract infections.

C. difficile produces toxins A and B, which contribute to the development of CDAD. Hypertoxin-producing strains of *C. difficile* cause increased morbidity and mortality, as these infections can be refractory to antimicrobial therapy and may require colectomy. CDAD must be considered in all patients who present with diarrhea following antibiotic use. Careful medical history is necessary because CDAD has been reported to occur more than 2 months after the administration of antibacterial agents.

If CDAD is suspected or confirmed, ongoing antibiotic use not directed against *C. difficile* may need to be discontinued. Institute appropriate fluid and electrolyte management, protein supplementation, antibiotic treatment of *C. difficile*, and surgical evaluation as clinically indicated.

Brand Names: US

Cleocin; Cleocin Phosphate

Brand Names: Canada

AG-Clindamycin [DSC]; Auro-Clindamycin; Dalacin C Palmitate; Dalacin C Phosphate; Dalacin C [DSC]; JAMP Clindamycin; M-Clindamycin; MED-Clindamycin; NRA-Clindamycin; RIVA-Clindamycin; TEVA-Clindamycin

Pharmacologic Category

Antibiotic, Lincosamide

Dosing: Adult

Usual dose:

Oral: 600 mg to 1.8 g/day in 2 to 4 divided doses.

IM, IV: 600 mg to 2.7 g/day in 2 to 4 divided doses; according to the manufacturer, up to 4.8 g/day IV (in divided doses) has been used in life-threatening infections; however, data supporting this dose are lacking; maximum: 600 mg/dose IM.

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Babesiosis



Babesiosis (alternative agent) (off-label use):

Mild to moderate disease: **Oral:** 600 mg every 8 hours in combination with quinine ([Ref](#)).

Severe disease, initial therapy: **IV:** 600 mg every 6 hours in combination with quinine; may switch to oral step-down therapy once symptoms improve ([Ref](#)).

Severe disease, oral step- down therapy: **Oral:** 600 mg every 8 hours in combination with quinine ([Ref](#)).

Duration of therapy: 7 to 10 days; a longer duration of ≥ 6 weeks, including 2 weeks after resolution of parasitemia, may be necessary for patients at high risk of relapse (eg, patients who are highly immunocompromised) ([Ref](#)).

Bacterial vaginosis



Bacterial vaginosis (alternative agent) (off-label use):

Note: Treatment is generally not warranted for patients who are asymptomatic ([Ref](#)).

Oral: 300 mg twice daily for 7 days ([Ref](#)).

Bite wound infection, prophylaxis or treatment

Bite wound infection, prophylaxis or treatment (animal or human bite)

(alternative agent) (off-label use): Note: For animal bite, use in combination with an appropriate agent for *Pasteurella multocida*. For human bite, use in combination with an appropriate agent for *Eikenella corrodens* ([Ref](#)).

Oral: 300 to 450 mg 3 times daily ([Ref](#)).

IV: 600 mg every 6 to 8 hours ([Ref](#)).

Duration: Duration is 3 to 5 days for prophylaxis; for established infection, continue for 1 to 2 days after resolution, typically 5 to 14 days total, although deep or complicated infection may require a longer course ([Ref](#)).

Diabetic foot infection, mild to moderate

Diabetic foot infection, mild to moderate (alternative agent):

Note: May be used alone for empiric therapy of mild infections; if there are risk factors for gram-negative bacilli, use in combination with other appropriate agents ([Ref](#)).

Oral: 300 mg 3 or 4 times daily or 450 mg 3 times daily ([Ref](#)). Duration of therapy depends on severity and clinical response. Most patients with mild infection limited to skin and soft tissue respond to 1 to 2 weeks of therapy; duration of therapy for moderate infection is usually 2 to 4 weeks in the absence of osteomyelitis ([Ref](#)).

Hidradenitis suppurativa

Hidradenitis suppurativa (off-label use): Oral: 300 mg twice daily, in combination with rifampin, for 10 to 12 weeks ([Ref](#)).

Malaria, treatment

Malaria, treatment (alternative agent) (off-label use): Oral: 20 mg/kg/day in divided doses every 8 hours for 7 days, as part of an appropriate combination regimen ([Ref](#)); oral doses generally do not exceed 1.8 g/day ([Ref](#)). **Note:** If used for severe malaria (after completion of IV therapy), use full 7-day schedule of clindamycin ([Ref](#)).

Mastitis, lactational

Mastitis, lactational (alternative agent):

Note: Reserve for patients unable to use first-line agents or for patients at risk for methicillin-resistant *S. aureus* ([Ref](#)).

Oral: 300 mg 4 times daily **or** 450 mg 3 times daily for 10 to 14 days; shorter courses (eg, 5 to 7 days) may be considered for patients with rapid clinical resolution ([Ref](#)).

Neutropenic fever, empiric therapy for low-risk patients with cancer

Neutropenic fever, empiric therapy for low-risk patients with cancer (alternative agent for patients with penicillin allergy) (off-label use):

Note: Reserve for patients with Multinational Association of Supportive Care in Cancer score ≥ 21 who did not receive fluoroquinolone prophylaxis ([Ref](#)).

Oral: 600 mg every 8 hours ([Ref](#)); some experts recommend 300 mg every 6 hours ([Ref](#)) (data on appropriate dose are limited). Use in combination with oral ciprofloxacin or levofloxacin; continue until resolution of fever and neutropenia or standard duration for the specific infection identified, if longer than the duration of neutropenia ([Ref](#)). Some experts consider discontinuation after ≥ 72 hours in patients who are hemodynamically stable and afebrile for at least 48 hours *without* a documented infection, regardless of marrow recovery ([Ref](#)). **Note:** Administer first dose in the health care setting (after blood cultures are drawn); observe patient for ≥ 4 hours before discharge ([Ref](#)).

Osteomyelitis and/or discitis

Osteomyelitis and/or discitis (alternative agent) :

IV: 600 to 900 mg every 8 hours for 6 weeks ([Ref](#)); for methicillin-resistant *S. aureus*, some experts use in combination with rifampin ([Ref](#)).

Oral: 300 to 450 mg 3 to 4 times daily ([Ref](#)) **or** 600 mg 3 times daily; for methicillin-resistant *S. aureus*, some experts use in combination with rifampin ([Ref](#)).

Duration of therapy: Duration is generally 6 weeks but depends on clinical response and presence of residual infection or hardware. Some patients who receive initial IV antibiotics can be transitioned to oral therapy to complete the course. Shorter courses are appropriate if the affected bone is completely resected by amputation ([Ref](#)).

Pelvic inflammatory disease, severe

Pelvic inflammatory disease, severe (including tubo-ovarian abscess) (alternative agent):

IV: 900 mg every 8 hours in combination with gentamicin; after 24 to 48 hours of sustained clinical improvement, transition to an oral regimen to complete 14 days of treatment ([Ref](#)). **Note:** Some experts reserve this regimen for patients who cannot use preferred agents due to greater associated adverse effects ([Ref](#)).

Oral: 450 mg 4 times daily, beginning after 24 to 48 hours of sustained clinical improvement on an appropriate parenteral regimen, to complete 14 days of therapy. **Note:** If tubo-ovarian abscess is present, use as part of an appropriate combination regimen ([Ref](#)).

Pneumocystis pneumonia, treatment

Pneumocystis pneumonia, treatment (alternative agent) (off-label use):

Mild to moderate disease: **Oral:** 450 mg every 6 hours **or** 600 mg every 8 hours, in combination with primaquine for 21 days ([Ref](#)).

Severe disease: **IV:** 600 mg every 6 hours **or** 900 mg every 8 hours, in combination with primaquine for 21 days; following clinical improvement, clindamycin can be given orally at 450 mg every 6 hours or 600 mg every 8 hours ([Ref](#)).

Note: Patients with moderate or severe infection ($\text{PaO}_2 < 70$ mm Hg at room air or alveolar-arterial oxygen gradient ≥ 35 mm Hg) should receive adjunctive glucocorticoids ([Ref](#)).

Pneumonia

Pneumonia

Aspiration pneumonia (alternative agent):

Note: Reserve for patients with penicillin allergy, as initial treatment for mild, community-acquired infection or as oral step-down therapy for patients requiring initial parenteral therapy ([Ref](#)).

Oral: 300 to 450 mg 3 times daily; duration of therapy is generally 5 to 7 days ([Ref](#)).

Pathogen-specific therapy for methicillin-resistant *S. aureus* pneumonia (alternative agent):

Note: Data in adults are limited ([Ref](#)).

Oral, IV: 600 mg 3 times daily; duration of therapy varies based on disease severity and response to therapy; treatment is typically given for 7 days ([Ref](#)).

Postpartum endometritis



Postpartum endometritis: IV: 900 mg every 8 hours in combination with gentamicin. Treat until the patient is clinically improved (no fundal tenderness) and afebrile for 24 to 48 hours ([Ref](#)). **Note:** For patients with group B *Streptococcus* colonization or sepsis, additional agents or other regimens are preferred ([Ref](#)).

Prosthetic joint infection



Prosthetic joint infection (off-label use):

Cutibacterium acnes, treatment (alternative agent for penicillin allergy):

IV: 600 to 900 mg every 8 hours for 4 to 6 weeks ([Ref](#)).

Oral: 300 to 450 mg every 6 hours ([Ref](#)), following at least 2 weeks of parenteral therapy ([Ref](#)).

Methicillin-resistant staphylococci, treatment (chronic suppression): Oral: 600 mg every 8 hours ([Ref](#)).

Rhinosinusitis, acute bacterial



Rhinosinusitis, acute bacterial (alternative agent) (off-label use):

Note: In uncomplicated acute bacterial rhinosinusitis, initial observation and symptom management without antibiotic therapy is appropriate in most patients. Reserve antibiotic therapy for patients with poor follow-up, risk factors for severe infection (eg, immunocompromising condition), or lack of improvement over the observation period. Reserve clindamycin for patients with penicillin allergy who are able to receive cephalosporins ([Ref](#)).

Oral: 300 mg every 6 to 8 hours in combination with an oral third-generation cephalosporin for 5 to 7 days ([Ref](#)).

Septic arthritis **Septic arthritis (alternative agent):**

Note: For use as pathogen-directed therapy for *S. aureus* (including MRSA) ([Ref](#)).

Oral, IV: 600 mg every 8 hours ([Ref](#)). Total treatment duration is 3 to 4 weeks (in the absence of osteomyelitis, bacteremia, or other complicating conditions), including appropriate oral step-down therapy ([Ref](#)).

Skin and soft tissue infection **Skin and soft tissue infection (alternative agent):*****Abscess:***

Note: Systemic antibiotics may be reasonably withheld in healthy, immunocompetent patients with a single, small abscess that has been drained if they are clinically well and have no indwelling device, risk factors for endocarditis, or risk for MRSA transmission ([Ref](#)).

Oral: 300 mg 4 times daily or 450 mg 3 times daily for ≥ 5 days; may extend up to 14 days depending on severity and clinical response ([Ref](#)).

Cellulitis, purulent or with risk for methicillin-resistant S. aureus or erysipelas:

Oral: 300 mg 4 times daily or 450 mg 3 times daily for ≥ 5 days; may extend up to 14 days depending on severity and clinical response ([Ref](#)).

IV: 600 to 900 mg every 8 hours ([Ref](#)).

Duration: Treat for ≥ 5 days but may extend up to 14 days depending on severity and clinical response ([Ref](#)).

Cellulitis, long-term suppression of recurrent infection: Note: For patients with recurrent presumptive staphylococcal cellulitis at the same anatomical site despite addressing predisposing factors; reserve for patients who cannot use other options ([Ref](#)).

Oral: 150 mg once daily after completion of treatment ([Ref](#)).

Impetigo or ecthyma if methicillin-resistant *S. aureus* is suspected or confirmed:

Note: For impetigo, reserve systemic therapy for patients with numerous lesions or in outbreak settings to decrease transmission ([Ref](#)).

Oral: 300 mg 4 times daily or 450 mg 3 times daily for 7 days ([Ref](#)).

Necrotizing soft tissue infection: IV: 600 to 900 mg every 8 hours as part of an appropriate combination regimen. **Note:** Antibiotic therapy must be used in conjunction with early and aggressive surgical exploration and debridement of necrotic tissue ([Ref](#)).

Streptococcus, group A



Streptococcus, group A:

Bloodstream infection: IV: 900 mg every 8 hours in combination with IV penicillin G; duration is individualized, but clindamycin may be discontinued within 48 hours for patients without septic shock, organ failure, or necrotizing infection. Continue an appropriate antistreptococcal agent to complete ≥ 14 days of therapy ([Ref](#)).

Pharyngitis (alternative agent for penicillin-allergic patients) (off-label use): Oral: 300 mg 3 times daily for 10 days ([Ref](#)).

Chronic carriage (off-label use): Oral: 300 mg 3 times daily for 10 days. **Note:** Most individuals with chronic carriage do not require antimicrobial treatment ([Ref](#)).

Streptococcus, group B, maternal prophylaxis for prevention of neonatal disease



Streptococcus, group B, maternal prophylaxis for prevention of neonatal disease (alternative agent) (off-label use):

IV: 900 mg at onset of labor or prelabor rupture of membranes, then every 8 hours until delivery. **Note:** Reserve use for patients with penicillin allergy that

are at high risk for anaphylaxis and who have documented clindamycin-susceptible group B streptococci ([Ref](#)).

Surgical prophylaxis

Surgical prophylaxis (off-label use):

Note: For use in combination with other appropriate agents when coverage for MRSA is indicated or for gram-positive coverage in patients unable to tolerate cephalosporins ([Ref](#)).

IV: 900 mg started within 60 minutes prior to initial surgical incision ([Ref](#)). May use in combination with azithromycin for nonelective cesarean delivery during labor or after membrane rupture ([Ref](#)). Clindamycin doses may be repeated intraoperatively at 6-hour intervals if procedure is lengthy or if there is excessive blood loss ([Ref](#)). In cases where an extension of prophylaxis is warranted postoperatively, total duration should be ≤ 24 hours ([Ref](#)). For clean and clean-contaminated procedures, continued prophylactic antibiotics beyond surgical incision closure is not recommended, even in the presence of a drain ([Ref](#)).

Toxic shock syndrome, toxin production suppression

Toxic shock syndrome, toxin production suppression: IV: 900 mg every 8 hours as part of an appropriate combination regimen ([Ref](#)). Duration is until clinically and hemodynamically stable for ≥ 48 to 72 hours; then discontinue clindamycin and give monotherapy with an appropriate agent ([Ref](#)).

Toxoplasma gondii encephalitis

Toxoplasma gondii encephalitis (alternative agent) (off-label use):

Initial treatment: **IV, Oral:** 600 mg every 6 hours in combination with pyrimethamine and leucovorin. Continue therapy for at least 6 weeks; longer duration may be required if incomplete response or extensive disease; after completion of acute therapy, all patients should receive chronic maintenance therapy ([Ref](#)).

Chronic maintenance therapy: **Oral:** 600 mg every 8 hours in combination with pyrimethamine and leucovorin. For patients with HIV, may discontinue when

asymptomatic with a CD4 count >200 cells/mm³ for >6 months in response to ART; for solid organ transplant recipients, continue lifelong ([Ref](#)).

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Adult

The renal dosing recommendations are based upon the best available evidence and clinical expertise. Senior Editorial Team: Bruce Mueller, PharmD, FCCP, FASN, FNKF; Jason A. Roberts, PhD, BPharm (Hons), B App Sc, FSHP, FISAC; Michael Heung, MD, MS.

Mild to severe impairment: IV, Oral: No dosage adjustment necessary ([Ref](#)).

Hemodialysis, intermittent (thrice weekly): IV, Oral: Poorly dialyzed; no supplemental dose or dosage adjustment necessary ([Ref](#)).

Peritoneal dialysis: IV, Oral: Poorly dialyzed; no dosage adjustment necessary ([Ref](#)).

CRRT: IV, Oral: No dosage adjustment necessary ([Ref](#)).

PIRRT (eg, sustained, low-efficiency diafiltration): IV, Oral: No dosage adjustment necessary ([Ref](#)).

Dosing: Liver Impairment: Adult

The liver dosing recommendations are based upon the best available evidence and clinical expertise. Senior Editorial Team: Matt Harris, PharmD, MHS, BCPS, FAST, FCCP; Jeong Park, PharmD, MS, BCTXP, FCCP, FAST; Arun Jesudian, MD; Sasan Sakiani, MD.

Liver impairment prior to treatment initiation:

Child-Turcotte-Pugh class A and B: No dosage adjustment necessary ([Ref](#)).

Child-Turcotte-Pugh class C: No dosage adjustment necessary. Use with caution; clindamycin accumulation may occur, especially if dosed more frequently than every 8 hours; however, the risk of underexposure generally outweighs the risk of accumulation ([Ref](#)).

Dosing: Older Adult

Refer to adult dosing.

Dosing: Pediatric

(For additional information see "[Clindamycin \(systemic\): Pediatric drug information](#)")

Dosage guidance:

Dosing: Dosing presented in mg/kg/dose and mg/kg/**day**; use caution. Dosage should be based on total body weight for children ≥ 2 years of age and adolescents with and without obesity ([Ref](#)).

General dosing:

Infants, Children, and Adolescents:

IM, IV: 20 to 40 mg/kg/**day** divided every 6 to 8 hours; maximum daily dose: 2,700 mg/**day** ([Ref](#)).

Oral: 10 to 25 mg/kg/**day** divided every 8 hours ([Ref](#)); higher doses of 30 to 40 mg/kg/**day** divided every 6 to 8 hours recommended for some infections, see specific indication; maximum daily dose: 1,800 mg/**day** ([Ref](#)).

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Anthrax



Anthrax: Limited data available:

Note: Consult public health officials for event-specific recommendations; do not delay therapy.

Postexposure prophylaxis: Infants, Children, and Adolescents: Oral: 10 mg/kg/dose every 8 hours; maximum dose: 600 mg/dose. Duration is based on type of exposure: For aerosol exposure, continue for 60 days; for nonaerosol exposure (ie, cutaneous or ingestion), continue for 7 days ([Ref](#)).

Cutaneous, without meningitis, treatment: Infants, Children, and Adolescents: Oral: 10 mg/kg/dose every 8 hours; maximum dose: 600 mg/dose. Treat for 7 to 10 days or until patient is clinically stable. If known or suspected aerosol exposure, continue for postexposure prophylaxis for a total of 60 days from exposure ([Ref](#)).

Systemic, including meningitis, treatment (alternative agent): Infants, Children, and Adolescents: IV: 13.3 mg/kg/dose every 8 hours; maximum dose: 900 mg/dose. Use as part of an appropriate combination regimen for ≥ 2 weeks; duration may be shortened and patient transitioned to oral therapy based on clinical response. Following completion of initial therapy, patients with a known or suspected aerosol exposure who are immunocompromised should transition to oral postexposure prophylaxis dosing for a total of 60 days from onset of illness ([Ref](#)).

Babesiosis



Babesiosis (alternative agent):

Infants, Children, and Adolescents: Limited data available:

Mild to moderate disease or oral step-down therapy following initial parenteral treatment: Oral: 7 to 10 mg/kg/dose every 6 to 8 hours in combination with quinine for a total of 7 to 10 days; maximum dose: 600 mg/dose. A longer duration of ≥ 6 weeks, including 2 weeks after resolution of parasitemia, may be necessary in highly immunocompromised patients ([Ref](#)).

Severe disease, initial therapy: IV: 7 to 10 mg/kg/dose every 6 to 8 hours in combination with quinine; maximum dose: 600 mg/dose; change to oral clindamycin once symptoms improve ([Ref](#)).

Endocarditis, prophylaxis before invasive dental or respiratory tract procedures



Endocarditis, prophylaxis before invasive dental or respiratory tract procedures:

Note: Clindamycin is NOT recommended for prevention of endocarditis in penicillin-allergic patients at high risk due to *C. difficile* risk; preferred options include cephalexin, azithromycin, clarithromycin, and doxycycline. Recommended only in patients who are at highest risk for infective endocarditis (IE) or adverse outcomes ([Ref](#)).

Infants, Children, and Adolescents: Limited data available: Oral, IV, IM: 20 mg/kg administered 30 to 60 minutes prior to procedure; maximum dose: 600 mg/dose ([Ref](#)).

Exit-site or tunnel infection, peritoneal dialysis catheter



Exit-site or tunnel infection, peritoneal dialysis catheter: Infant, Children, and Adolescents: Oral: 10 mg/kg/dose 3 times daily; maximum dose: 450 mg/dose ([Ref](#)).

Intra-abdominal infection, complicated



Intra-abdominal infection, complicated: Note: Not routinely recommended due to *Bacteroides fragilis* resistance ([Ref](#)).

Infants, Children, and Adolescents: IV: 20 to 40 mg/kg/**day** divided every 6 to 8 hours in combination with other antibiotics ([Ref](#)); maximum daily dose: 2,700 mg/**day** ([Ref](#)).

Malaria, uncomplicated, treatment

Malaria, uncomplicated, treatment (alternative agent): Infants, Children, and Adolescents: Oral: 20 mg/kg/**day** divided every 8 hours for 7 days in combination with quinine ([Ref](#)).

Osteoarticular infection, acute

Osteoarticular infection, acute (eg, bacterial arthritis, osteomyelitis): Infants, Children, and Adolescents: IV, Oral: 30 to 40 mg/kg/**day** divided every 6 to 8 hours; maximum dose: IV: 900 mg/dose; Oral: 600 mg/dose. Duration should be individualized based on several factors including causative pathogen, response to therapy, and normalization of inflammatory markers. For acute hematogenous osteomyelitis, the minimum total duration is 3 to 4 weeks of therapy. For acute bacterial arthritis, typical duration is 3 to 4 weeks, but courses as short as 10 to 14 days may be considered in patients with adequate source control who improve rapidly with a consistent, progressive decrease in C-reactive protein by the end of the first week. Longer duration commonly necessary, particularly for infections caused by methicillin-resistant *Staphylococcus aureus* (MRSA) ([Ref](#)).

Otitis media, acute

Otitis media, acute (alternative agent): Note: Typically reserved for patients who cannot tolerate beta-lactam antibiotics or as an alternative in patients in whom initial therapy for acute otitis media (AOM) fails. In some instances, it may be necessary to use as part of a combination regimen (eg, when activity is desired against *Haemophilus influenzae* or *Moraxella catarrhalis*) ([Ref](#)).

Infants ≥ 6 months, Children, and Adolescents: Oral: 30 to 40 mg/kg/**day** divided every 6 to 8 hours; maximum daily dose: 1,800 mg/**day** ([Ref](#)). For patients with severe or recurrent AOM, tympanic membrane perforation, or who are < 2 years of age, treat for 10 days; for patients ≥ 2 years of age with mild to moderate, non-recurrent disease without tympanic membrane perforation, shorter durations of 5 to 7 days may be sufficient ([Ref](#)).

Peritonitis



Peritonitis (peritoneal dialysis):

Treatment: Infants, Children, and Adolescents: Intraperitoneal, continuous: Loading dose: 300 mg per liter of dialysate; maintenance dose: 125 to 150 mg per liter of dialysate ([Ref](#)). **Note:** Intraperitoneal clindamycin is no longer included in guidelines for treatment of peritonitis ([Ref](#)).

Pneumocystis jirovecii pneumonia, treatment



Pneumocystis jirovecii pneumonia (PCP), treatment (alternative agent):

HIV-exposed/-infected: **Note:** Patients with moderate or severe infection ($\text{PaO}_2 < 70$ mm Hg at room air or alveolar-arterial oxygen gradient ≥ 35 mm Hg) should receive adjunctive glucocorticoids ([Ref](#)).

Infants and Children: Limited data available: IV, Oral: 10 mg/kg/dose every 6 hours in combination with primaquine for 21 days; maximum IV dose: 600 mg/dose; maximum oral dose: 300 to 450 mg/dose ([Ref](#)).

Adolescents ([Ref](#)):

Oral: Mild to severe disease: 450 mg every 6 hours **or** 600 mg every 8 hours in combination with primaquine for 21 days.

IV: Moderate to severe disease: 600 mg every 6 hours **or** 900 mg every 8 hours in combination with primaquine for 21 days.

Pneumonia, community acquired



Pneumonia, community acquired: **Note:** Duration dependent upon pathogen and clinical course. Typical duration for uncomplicated infections is 5 to 10 days; however, infections caused by MRSA may require longer treatment ([Ref](#)).

Infants ≥ 3 months, Children, and Adolescents ([Ref](#)):

Moderate to severe infection: IV: 40 mg/kg/**day** divided every 6 to 8 hours; maximum daily dose: 2,700 mg/**day**.

Mild infection or oral step-down therapy: Oral: 30 to 40 mg/kg/**day** divided every 6 to 8 hours; maximum daily dose: 1,800 mg/**day**.

Rhinosinusitis, acute bacterial



Rhinosinusitis, acute bacterial (alternative agent): Children and Adolescents: Oral: 30 to 40 mg/kg/**day** divided every 8 hours for 10 to 14 days ([Ref](#)). **Note:** Use as part of appropriate combination therapy if activity against *H. influenzae* or *M. catarrhalis* is desired ([Ref](#)).

Skin and soft tissue infection



Skin and soft tissue infection (SSTI):

Infants, Children, and Adolescents:

Impetigo, ecthyma (if MRSA is suspected or confirmed): Oral: 20 mg/kg/**day** in divided doses every 8 hours for 7 days; maximum dose: 400 mg/dose ([Ref](#)).

Cellulitis, erysipelas, purulent/fluctuant SSTI: **Note:** Typical duration is 5 days for uncomplicated infection but may be extended if clinical response is inadequate ([Ref](#)).

IV: 25 to 40 mg/kg/**day** in divided doses every 8 hours; maximum dose: 600 mg/dose ([Ref](#)).

Oral:

Methicillin-susceptible *Staphylococcus aureus* (MSSA) infection: Oral: 25 to 30 mg/kg/**day** in divided doses every 8 hours; maximum dose: 450 mg/dose ([Ref](#)).

MRSA infection: Oral: 30 to 40 mg/kg/**day** in divided doses every 6 to 8 hours; maximum dose: 450 mg/dose ([Ref](#)).

Necrotizing soft tissue infections: IV: 10 to 13 mg/kg/dose every 8 hours as part of an appropriate combination regimen in addition to surgical intervention; maximum dose: 900 mg/dose. Continue until further debridement is not necessary, patient has clinically improved, and patient is afebrile for 48 to 72 hours ([Ref](#)).

Streptococcus, group A



Streptococcus, group A:

Pharyngitis/tonsillitis (alternative agent for severe penicillin allergy): Children and Adolescents: Oral: 21 mg/kg/**day** in divided doses every 8 hours for 10 days; maximum dose: 300 mg/dose ([Ref](#)).

*Chronic carriage: **Note:*** Most individuals with chronic carriage do not require antibiotic treatment ([Ref](#)).

Children and Adolescents: Oral: 20 to 30 mg/kg/**day** in divided doses every 8 hours for 10 days; maximum dose: 300 mg/dose ([Ref](#)).

Surgical prophylaxis

Surgical prophylaxis: Children and Adolescents: IV: 10 mg/kg within 30 to 60 minutes prior to procedure; may repeat dose in 6 hours for prolonged procedure or excessive blood loss; maximum dose: 900 mg/dose ([Ref](#)).

Toxic shock syndrome, toxin production suppression

Toxic shock syndrome, toxin production suppression (empiric therapy): Infants, Children, and Adolescents: IV: 40 mg/kg/**day** in divided doses every 6 to 8 hours; maximum dose: 900 mg/dose ([Ref](#)).

Toxoplasmosis

Toxoplasmosis (alternative agent):

HIV-exposed/-infected: Infants, Children, and Adolescents:

Initial treatment: IV, Oral: 5 to 7.5 mg/kg/dose every 6 hours in combination with pyrimethamine and leucovorin; maximum dose: 600 mg/dose. For congenital toxoplasmosis, continue therapy for 12 months. For acquired toxoplasmosis, continue for ≥ 6 weeks followed by chronic maintenance therapy; longer duration may be required if incomplete response or extensive disease ([Ref](#)).

Chronic maintenance therapy (secondary prophylaxis): Oral: 7 to 10 mg/kg/dose every 8 hours in combination with pyrimethamine and leucovorin; maximum dose: 600 mg/dose. May consider discontinuation when asymptomatic, CD4 percentage is $\geq 15\%$ (or CD4 count is >200 cells/mm³ for ages ≥ 6 years), and the patient has an undetectable HIV viral load in response to antiretroviral therapy for ≥ 6 months ([Ref](#)).

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Pediatric

Altered kidney function: Infants, Children, and Adolescents: IV, Oral:

Mild to severe impairment: No dosage adjustment necessary.

Hemodialysis, intermittent (thrice weekly): Poorly dialyzed; based on adult information, no supplemental dose or dosage adjustment necessary ([Ref](#)).

Peritoneal dialysis: Poorly dialyzed; based on adult information, no dosage adjustment necessary ([Ref](#)).

Continuous renal replacement therapy (CRRT): Based on adult information, no dosage adjustment necessary ([Ref](#)).

Dosing: Liver Impairment: Pediatric

No adjustment required. Use caution with severe hepatic impairment.

Adverse Reactions (Significant): Considerations

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Antibiotic-associated (non-*Clostridioides difficile*) diarrhea

GI effects range from antibiotic-associated [non-*C. difficile*] **diarrhea** (AAD), **nausea**, and **vomiting**. Most cases of AAD are mild and self-limiting. However, *Clostridioides difficile* may account for as many as >20% of cases in children, adolescents, and adults (discussed separately) and result in more severe AAD ([Ref](#)).

Mechanism: Dose- and time-related; antibiotic disruption of indigenous gut microbiota ([Ref](#)).

Onset: Varied; mean time to onset of AAD is 3 to 18 days for adult patients and 2 to 6 days for pediatric patients. The majority of AAD cases occur during (versus after) antibiotic therapy in pediatric patients ([Ref](#)).

Risk factors:

- Duration of therapy ([Ref](#))
- Age (pediatric patients <2 years of age and older adults) ([Ref](#))
- Length of hospitalization or ICU stay ([Ref](#))

- Duration of proton pump inhibitor use ([Ref](#))
- Parenteral nutrition ([Ref](#))
- Combinations of antibiotics ([Ref](#))

Clostridioides difficile infection



***Clostridioides difficile* infection** (CDI) has occurred, including ***Clostridioides difficile* –associated diarrhea** (CDAD) and ***Clostridioides difficile* colitis**. Clindamycin has been associated with a several fold increased risk of CDI ([Ref](#)). CDAD must be considered in all patients who present with diarrhea following antibiotic use. If CDAD is suspected or confirmed, ongoing antibiotic use not directed against *C. difficile* should be discontinued if possible. Institute appropriate fluid and electrolyte management, antibiotic treatment of *C. difficile*, and surgical evaluation as clinically indicated.

Mechanism: Dose- and time-related; related to cumulative antibiotic exposure. Clindamycin causes disruption of the intestinal microbiota resulting in the overgrowth of pathogens, such as *C. difficile* ([Ref](#)). In addition, *C. difficile* is highly resistant to clindamycin ([Ref](#)).

Onset: Varied; may start on the first day of antibiotic therapy or up to 3 months postantibiotic ([Ref](#)).

Risk factors:

- Antibiotic exposure (highest risk factor) ([Ref](#))
- Type of antibiotic (clindamycin among the highest risk) ([Ref](#))
- Long durations in a hospital or other health care setting (recent or current) ([Ref](#))
- Older adults ([Ref](#))
- Immunocompromised conditions ([Ref](#))
- A serious underlying condition ([Ref](#))
- GI surgery/manipulation ([Ref](#))
- Antiulcer medications (eg, proton pump inhibitors and H2 blockers) ([Ref](#))
- Chemotherapy ([Ref](#))

Hypersensitivity reactions (immediate and delayed)



Hypersensitivity reactions (immediate and delayed) range from **maculopapular rash** to rare cases of **anaphylaxis** ([Ref](#)). Severe cutaneous adverse reactions (SCARs), including **drug reaction with eosinophilia and systemic symptoms, acute generalized exanthematous pustulosis, Stevens-Johnson syndrome, symmetrical drug-related intertriginous and flexural exanthema**, and **toxic epidermal necrolysis** have been reported ([Ref](#)). **Ischemic heart disease** with or without **acute myocardial infarction** occurring as part of an allergic reaction has also been reported.

Mechanism: Non-dose-related; immunologic.

Immediate hypersensitivity reactions (eg, anaphylaxis): IgE-mediated ([Ref](#)).

Delayed hypersensitivity reactions (eg, maculopapular rash, SCARs), T-cell-mediated ([Ref](#)).

Onset:

Immediate hypersensitivity reactions (eg, anaphylaxis): Rapid; may occur within an hour of administration ([Ref](#)).

Delayed hypersensitivity reaction: Maculopapular rash: Intermediate; usually 7 to 10 days after initiation ([Ref](#)). Other reactions: Varied; may occur after days to weeks of therapy ([Ref](#)).

Adverse Reactions

The following adverse drug reactions are derived from product labeling unless otherwise specified.

Frequency not defined:

Dermatologic: Urticaria, vesiculobullous dermatitis

Gastrointestinal: Abdominal pain, nausea, vomiting

Postmarketing:

Cardiovascular: Acute myocardial infarction, hypotension (following rapid IV administration), ischemic heart disease, thrombophlebitis (IV)

Dermatologic: Acute generalized exanthematous pustulosis ([Ref](#)), dermatologic reaction (symmetrical drug-related intertriginous and flexural exanthema [SDRIFE]) ([Ref](#)), erythema multiforme, exfoliative dermatitis, fixed drug eruption ([Ref](#)),

maculopapular rash ([Ref](#)), Stevens-Johnson syndrome, Sweet syndrome ([Ref](#)), toxic epidermal necrolysis ([Ref](#))

Gastrointestinal: *Clostridioides difficile*-associated diarrhea ([Ref](#)), *Clostridioides difficile* colitis ([Ref](#)), diarrhea ([Ref](#)), dysgeusia ([Ref](#)), esophageal ulcer ([Ref](#)), esophagitis ([Ref](#))

Genitourinary: Vaginitis

Hematologic & oncologic: Agranulocytosis ([Ref](#)), eosinophilia (transient), neutropenia (transient) ([Ref](#)), pancytopenia ([Ref](#)), thrombocytopenia ([Ref](#))

Hepatic: Cholestatic hepatitis ([Ref](#))

Hypersensitivity: Anaphylactic shock ([Ref](#)), anaphylaxis ([Ref](#)), angioedema, drug reaction with eosinophilia and systemic symptoms ([Ref](#)), hypersensitivity angitis ([Ref](#))

Local: Abscess at injection site (IM), induration at injection site (IM), irritation at injection site (IM), pain at injection site (IM)

Neuromuscular & skeletal: Inflammatory polyarthritis

Renal: Acute kidney injury ([Ref](#))

Contraindications

Hypersensitivity to clindamycin, lincomycin, or any component of the formulation.

Canadian labeling: Additional contraindications (not in US labeling): Oral clindamycin: Infants <30 days of age.

Warnings/Precautions

Concerns related to adverse effects:

- Renal toxicity: Acute kidney injury has been reported; discontinue treatment if clindamycin-induced acute kidney injury is suspected and no other etiology is identified.
- Superinfection: Use may result in overgrowth of nonsusceptible organisms, particularly yeast. Should superinfection occur, appropriate measures should be taken as indicated by the clinical situation.

Disease-related concerns:

- GI disease: Use with caution in patients with a history of GI disease, particularly colitis.
- Liver impairment: Use with caution in patients with moderate to severe liver disease; however, when administered at every-8-hour intervals, drug accumulation is rare.

- Renal impairment: Use with caution in patients with renal impairment; acute kidney injury may occur, especially if patient is taking other nephrotoxins concurrently.

Special populations:

- Atopic patients: Use with caution in atopic patients.
- Older adult: A subgroup of older patients with associated severe illness may tolerate diarrhea less well. Monitor carefully for changes in bowel frequency.

Dosage form specific issues:

- Benzyl alcohol and derivatives: Some dosage forms may contain benzyl alcohol; large amounts of benzyl alcohol (≥ 99 mg/kg/day) have been associated with a potentially fatal toxicity ("gasping syndrome") in neonates; the "gasping syndrome" consists of metabolic acidosis, respiratory distress, gasping respirations, CNS dysfunction (including convulsions, intracranial hemorrhage), hypotension and cardiovascular collapse (AAP ["Inactive" 1997]; CDC 1982); some data suggests that benzoate displaces bilirubin from protein binding sites (Ahlfors 2001); avoid or use dosage forms containing benzyl alcohol with caution in neonates. See manufacturer's labeling.
- Capsules: Administer with a full glass of water (approximately 200 to 250 mL [6 to 8 ounces]); ensure patient remains in an upright position ≥ 30 minutes following dose to avoid esophageal irritation.
- Tartrazine: Some products may contain tartrazine (FD&C yellow no. 5), which may cause allergic reactions in certain individuals. Allergy is frequently seen in patients who also have an aspirin hypersensitivity.

Other warnings/precautions:

- Administration (IV): Do not inject IV undiluted as a bolus. Product should be diluted in compatible fluid and infused over 10 to 60 minutes.
- Appropriate use: Not appropriate for use in the treatment of meningitis due to inadequate penetration into the CSF.

Dosage Forms: US

Excipient information presented when available (limited, particularly for generics); consult specific product labeling. [DSC] = Discontinued product

Capsule, Oral, as hydrochloride [strength expressed as base]:

Cleocin: 75 mg, 150 mg [contains fd&c blue #1 (brilliant blue), fd&c yellow #5 (tartrazine)]

Cleocin: 300 mg [contains fd&c blue #1 (brilliant blue)]

Generic: 75 mg, 150 mg, 300 mg

Solution, Injection, as phosphate [strength expressed as base]:

Cleocin Phosphate: 300 mg/2 mL (2 mL); 600 mg/4 mL (4 mL); 900 mg/6 mL (6 mL); 9 g/60 mL (60 mL) [contains benzyl alcohol, edetate (edta) disodium]

Generic: 300 mg/2 mL (2 mL); 600 mg/4 mL (4 mL); 900 mg/6 mL (6 mL); 9 g/60 mL (60 mL) [DSC])

Solution, Intravenous, as phosphate [strength expressed as base]:

Generic: 600 mg/50 mL in D5W (50 mL); 900 mg/50 mL in D5W (50 mL)

Solution, Intravenous, as phosphate [strength expressed as base, preservative free]:

Generic: 300 mg/50 mL in D5W (50 mL); 300 mg/50 mL in NaCl 0.9% (50 mL); 600 mg/50 mL in D5W (50 mL); 600 mg/50 mL in NaCl 0.9% (50 mL); 900 mg/50 mL in D5W (50 mL); 900 mg/50 mL in NaCl 0.9% (50 mL)

Solution Reconstituted, Oral, as palmitate hydrochloride [strength expressed as base]:

Cleocin: 75 mg/5 mL (100 mL) [contains ethylparaben (ethyl p-hydroxybenzoate); cherry flavor]

Generic: 75 mg/5 mL (100 mL)

Generic Equivalent Available: US

Yes

Pricing: US

Capsules (Cleocin Oral)

75 mg (per each): \$0.27

150 mg (per each): \$0.17

300 mg (per each): \$0.34

Capsules (Clindamycin HCl Oral)

75 mg (per each): \$0.72 - \$3.60

150 mg (per each): \$0.18 - \$4.20

300 mg (per each): \$0.54 - \$4.80

Solution (Cleocin Phosphate Injection)

9 g/60 mL (per mL): \$0.49

300 mg/2 mL (per mL): \$1.49

600 mg/4 mL (per mL): \$0.88

900 mg/6 mL (per mL): \$0.85

Solution (Clindamycin Phosphate in D5W Intravenous)

300 mg/50 mL (per mL): \$0.15 - \$0.20

600 mg/50 mL (per mL): \$0.10 - \$0.30

900 mg/50 mL (per mL): \$0.12 - \$0.36

Solution (Clindamycin Phosphate in NaCl Intravenous)

300 mg/50 mL 0.9% (per mL): \$0.17

600 mg/50 mL 0.9% (per mL): \$0.26

900 mg/50 mL 0.9% (per mL): \$0.31

Solution (Clindamycin Phosphate Injection)

300 mg/2 mL (per mL): \$2.05

600 mg/4 mL (per mL): \$1.21

900 mg/6 mL (per mL): \$1.09

Solution (reconstituted) (Cleocin Oral)

75 mg/5 mL (per mL): \$0.39

Solution (reconstituted) (Clindamycin Palmitate HCl Oral)

75 mg/5 mL (per mL): \$0.62 - \$1.08

Disclaimer: A representative AWP (Average Wholesale Price) price or price range is provided as reference price only. A range is provided when more than one manufacturer's AWP price is available and uses the low and high price reported by the manufacturers to determine the range. The pricing data should be used for benchmarking purposes only, and as such should not be used alone to set or adjudicate any prices for reimbursement or purchasing functions or considered to be an exact price for a single product and/or manufacturer. Medi-Span expressly disclaims all warranties of any kind or nature, whether express or implied, and assumes no liability with respect to accuracy of price or price range data published in its

solutions. In no event shall Medi-Span be liable for special, indirect, incidental, or consequential damages arising from use of price or price range data. Pricing data is updated monthly.

Dosage Forms: Canada

Excipient information presented when available (limited, particularly for generics); consult specific product labeling. [DSC] = Discontinued product

Capsule, Oral, as hydrochloride [strength expressed as base]:

Dalacin C: 150 mg [DSC], 300 mg [DSC]

Generic: 150 mg, 300 mg

Solution, Injection:

Dalacin C Phosphate: 150 mg/mL (2 mL, 4 mL, 6 mL, 60 mL) [contains benzyl alcohol, edetate (edta) disodium]

Generic: 150 mg/mL (2 mL, 4 mL, 6 mL, 60 mL, 120 mL)

Solution, Intravenous, as phosphate [strength expressed as base]:

Generic: 300 mg/50 mL in D5W ([DSC]); 600 mg/50 mL in D5W (50 mL); 900 mg/50 mL in D5W (50 mL)

Solution Reconstituted, Oral, as palmitate hydrochloride [strength expressed as base]:

Dalacin C Palmitate: 75 mg/5 mL (100 mL) [contains ethylparaben (ethyl p-hydroxybenzoate)]

Administration: Adult

IM: Administer undiluted deep IM; rotate sites. Do not exceed 600 mg in a single injection.

IV: Never administer undiluted as bolus; administer by IV intermittent infusion over at least 10 to 60 minutes, at a maximum rate of 30 mg/minute (do not exceed 1.2 g/hour).

Oral: Administer capsule with a full glass of water (6 to 8 ounces) at least 30 minutes prior to lying down to avoid potential of esophageal irritation; shake oral solution well before use and measure using an accurate measuring device (eg, oral syringe); may administer with or without meals.

Enteral feeding tube:

The following recommendations are based upon the best available evidence and clinical expertise. Senior editorial team: Joseph I. Boullata, PharmD, RPh, CNS-S, FASPEN,

FACN; Peggi A. Guenter PhD, RN, FASPEN; Kathleen Gura, PharmD, BCNSP, FASHP, FASPEN, FPPA, FMSHP; Mark G. Klang MS, RPh, BCNSP, PhD, FASPEN; Linda Lord, NP, ACNP-BC, CNSC, FASPEN.

Oral capsule:

Gastric (eg, NG, G-tube) or post-pyloric (eg, J-tube) tubes: Open and disperse contents of each capsule in 10 to 15 mL purified water; draw up mixture into enteral dosing syringe and administer via feeding tube ([Ref](#)).

General guidance: Hold enteral nutrition during clindamycin administration ([Ref](#)). Flush feeding tube with an appropriate volume of purified water (eg, 15 mL) before administration ([Ref](#)). Following administration, rinse container used for preparation with purified water (eg, 15 mL); draw up rinse and administer contents to ensure delivery of entire dose ([Ref](#)). Flush feeding tube with an appropriate volume of purified water (eg, 15 mL) and restart enteral nutrition ([Ref](#)).

Oral solution (commercially available):

Gastric (eg, NG, G-tube) or post-pyloric (eg, J-tube) tubes: Shake solution well prior to drawing up dose for dilution. Dilute dose with at least an equivalent volume of purified water immediately prior to administration to reduce osmolality and viscosity. Draw up diluted solution into enteral dosing syringe and administer via feeding tube ([Ref](#)).

Dosage form information: One undiluted formulation has been reported to have an osmolality of ~1,200 mOsm/kg ([Ref](#)); oral solutions with an osmolality >600 mOsm/kg may increase the probability of adverse GI effects (eg, diarrhea, cramping, abdominal distention, slowed gastric emptying), particularly if administered post-pylorically, inadequately diluted, and/or used in at-risk patients (eg, neonates and infants, patients with short bowel syndrome) ([Ref](#)).

General guidance: Hold enteral nutrition during clindamycin administration ([Ref](#)). Flush feeding tube with an appropriate volume of purified water (eg, 15 mL) before administration ([Ref](#)). Following administration, rinse container used for preparation with purified water; draw up rinse and administer contents to ensure delivery of entire dose ([Ref](#)). Flush feeding tube with an appropriate volume of purified water (eg, 15 mL) and restart enteral nutrition ([Ref](#)).

No te: Recommendations may not account for differences in inactive ingredients, osmolality, or other formulation properties that may vary among products from

different manufacturers.

Preparation for Administration: Adult

Injection: Never administer undiluted as bolus. For IV infusion, dilute vials requiring dilution with 50 to 100 mL of compatible diluent (eg, D5W, NS); concentration of clindamycin for IV infusion should not exceed 18 mg/mL. **Note:** Premixed bags and vials are also available.

Oral solution: Reconstitute bottles of 100 mL with 75 mL of water. Add a large portion of the water and shake vigorously; add the remainder of the water and shake until the solution is uniform. When reconstituted with water, each 5 mL of solution contains clindamycin palmitate hydrochloride equivalent to clindamycin 75 mg.

Administration: Pediatric

The following feeding tube recommendations are based upon the best available evidence and clinical expertise. Senior editor panel: Joseph I. Boullata, PharmD, RPh, CNS-S, FASPEN, FACN; Peggi A. Guenter, PhD, RN, FASPEN; Kathleen Gura, PharmD, BCNSP, FASHP, FASPEN, FPPA, FMSHP; Mark G. Klang, MS, RPh, BCNSP, PhD, FASPEN; Linda Lord, NP, ACNP-BC, CNSC, FASPEN.

Note: Recommendations may not account for differences in inactive ingredients, osmolality, or other formulation properties that may vary among products from different manufacturers.

Oral: May administer with or without meals.

Capsule: Administer with a full glass of water (approximately 200 to 250 mL [6 to 8 ounces]); ensure patient remains in an upright position ≥ 30 minutes following dose to avoid esophageal irritation.

Administration via feeding tube:

Gastric (eg, NG, G-tube) or post-pyloric tubes (eg, J-tube): Open and disperse contents of each capsule in 10 to 15 mL purified water; draw up mixture into enteral dosing syringe and administer via feeding tube ([Ref](#)).

General guidance: Hold enteral nutrition during clindamycin administration ([Ref](#)). Flush feeding tube with the lowest volume of purified water necessary to clear the tube prior to administration based on size of patient and/or feeding tube (eg, neonates: 1 to 3 mL; infants and children: 2 to 5 mL; adolescents: 15 mL); refer to institutional policies and procedures ([Ref](#)). Following administration, rinse container used for preparation with purified water; draw up rinse and administer contents

to ensure delivery of entire dose ([Ref](#)). Flush feeding tube with an appropriate volume of purified water and restart enteral nutrition ([Ref](#)).

Oral solution: (commercially available): Shake well before use; administer with an accurate measuring device (eg, calibrated oral syringe or measuring cup); do not use a household teaspoon or tablespoon to measure dose (overdosage may occur).

Administration via feeding tube:

Gastric (eg, NG, G-tube) or post-pyloric tubes (eg, J-tube): Shake solution well prior to drawing up dose for dilution. Dilute dose with at least an equivalent volume of purified water immediately prior to administration to reduce osmolality and viscosity. Draw up diluted solution into enteral dosing syringe and administer via feeding tube ([Ref](#)).

Dosage form information: One commercially available oral solution has a reported osmolality of ~1200 mOsm/kg when undiluted ([Ref](#)); oral solutions with an osmolality >600 mOsm/kg may increase the probability of adverse GI effects (eg, diarrhea, cramping, abdominal distention, slowed gastric emptying), particularly if administered post-pylorically, inadequately diluted, and/or used in at-risk patients (eg, neonates and infants, patients with short bowel syndrome) ([Ref](#)).

General guidance: Hold enteral nutrition during clindamycin administration ([Ref](#)). Flush feeding tube with the lowest volume of purified water necessary to clear the tube prior to administration based on size of patient and/or feeding tube (eg, neonates: 1 to 3 mL; infants and children: 2 to 5 mL; adolescents: 15 mL); refer to institutional policies and procedures ([Ref](#)). Following administration, rinse container used for preparation with purified water; draw up rinse and administer contents to ensure delivery of entire dose ([Ref](#)). Flush feeding tube with an appropriate volume of purified water and restart enteral nutrition ([Ref](#)).

Parenteral:

IM: Administer undiluted deep IM; rotate sites. Do not exceed 600 mg in a single injection.

IV: Infuse over at least 10 to 60 minutes, at a rate not to exceed 30 mg/minute; hypotension and cardiopulmonary arrest have been reported following rapid IV administration.

Preparation for Administration: Pediatric

Oral: Reconstitute 100 mL bottles of powder for oral solution with 75 mL of water. Add a large portion of the water and shake vigorously; add the remainder of the water and shake until suspended. When reconstituted, each 5 mL of solution contains clindamycin palmitate hydrochloride equivalent to clindamycin 75 mg.

Parenteral: IV: Dilute vials that require dilution (150 mg/mL) with a compatible diluent (eg, D5W, NS) to a final concentration ≤ 18 mg/mL. For neonatal patients, Vermont Oxford Network recommends a concentration of 6 mg/mL ([Ref](#)). **Note:** Premixed bags and vials are also available (6, 12, and 18 mg/mL).

Storage/Stability

Oral: Store at 20°C to 25°C (68°F to 77°F). Do not refrigerate the reconstituted oral solution (it will thicken); the solution is stable for 2 weeks at room temperature.

IV: Store intact vials and premixed bags at 20°C to 25°C (68°F to 77°F). Solutions diluted for infusion in NS or D5W are stable for 16 days at room temperature, 32 days refrigerated, or 8 weeks frozen. After initial use, discard any unused portion of the pharmacy bulk package (9 g per 60 mL) vial after 24 hours.

Use: Labeled Indications

Bone and joint infections: Treatment of bone and joint infections, including acute hematogenous osteomyelitis caused by *Staphylococcus aureus* and as adjunctive therapy in the surgical treatment of chronic bone and joint infections caused by susceptible organisms.

Gynecological infections: Treatment of gynecologic infections, including endometritis, nongonococcal tubo-ovarian abscess, pelvic cellulitis, and postsurgical vaginal cuff infection caused by susceptible anaerobes.

Intraabdominal infections: Treatment of intraabdominal infections, including peritonitis and intraabdominal abscess caused by susceptible anaerobic organisms.

Lower respiratory tract infections: Treatment of lower respiratory tract infections, including pneumonia, empyema, and lung abscess caused by susceptible anaerobes, *Streptococcus pneumoniae*, other streptococci (except *Enterococcus faecalis*), and *S. aureus*.

Septicemia: Treatment of septicemia caused by *S. aureus*, streptococci (except *E. faecalis*), and susceptible anaerobes.

Skin and soft tissue infection: Treatment of skin and soft tissue infection caused by *Streptococcus pyogenes*, *S. aureus*, and susceptible anaerobes.

Use: Off-Label: Adult

Babesiosis; Bacterial vaginosis; Bite wound infection, prophylaxis or treatment; Hidradenitis suppurativa; Malaria, treatment; Neutropenic fever, empiric therapy for low-risk patients with cancer; Pneumocystis pneumonia, treatment; Prosthetic joint infection; Rhinosinusitis, acute bacterial; Streptococcal (group A) pharyngitis and chronic carriage; Streptococcus (group B), maternal prophylaxis for prevention of neonatal disease; Surgical prophylaxis; Toxoplasma gondii encephalitis, treatment/chronic maintenance

Medication Safety Issues

[Collapse All](#)

Sound-alike/look-alike issues:

Cleocin may be confused with bleomycin, Clinoril, Cubicin, Lincocin

Clindamycin may be confused with clarithromycin, Claritin, vancomycin, lincomycin

High alert medication:

The Institute for Safe Medication Practices (ISMP) includes this medication among its list of drugs (pediatric liquid medications requiring measurement) which have a heightened risk of causing significant patient harm when used in error (ISMP List of High-Alert Medications in Community/Ambulatory Care Settings ©ISMP 2021).

International issues:

Dynacin [Philippines] may be confused with Dynacirc brand name for isradipine [multiple international markets]

Metabolism/Transport Effects

Substrate of CYP3A4 (Major); **Note:** Assignment of Major/Minor substrate status based on clinically relevant drug interaction potential

Drug Interactions

(For additional information: [Launch drug interactions program](#))

Note: Interacting drugs may **not be individually listed below** if they are part of a group interaction (eg, individual drugs within “CYP3A4 Inducers [Strong]” are NOT listed). For a complete list of drug interactions by individual drug name and detailed management recommendations, use the drug interactions program by clicking on the “Launch drug interactions program” link above.

Aescin: Antibiotics may increase serum concentrations of Aescin. Management: Consider alternatives to this combination as aescin product labeling states coadministration with antibiotics is not recommended. *Risk D: Consider Therapy Modification*

Bacillus Calmette Guerin (BCG) (Intravesical): Antibiotics may decrease therapeutic effects of Bacillus Calmette Guerin (BCG) (Intravesical). *Risk X: Avoid*

Bacillus Calmette Guerin (BCG) (Percutaneous): Antibiotics may decrease therapeutic effects of Bacillus Calmette Guerin (BCG) (Percutaneous). *Risk C: Monitor*

Bacillus clausii: Antibiotics may decrease therapeutic effects of Bacillus clausii. Management: Bacillus clausii should be taken in between antibiotic doses during concomitant therapy. *Risk D: Consider Therapy Modification*

Cholera Vaccine: Antibiotics may decrease therapeutic effects of Cholera Vaccine. Management: Avoid cholera vaccine in patients receiving systemic antibiotics, and within 14 days following the use of oral or parenteral antibiotics. *Risk X: Avoid*

Clofazimine: May increase serum concentrations of CYP3A4 Substrates (High risk with Inhibitors). *Risk C: Monitor*

CYP3A4 Inducers (Moderate): May decrease serum concentrations of Clindamycin (Systemic). *Risk C: Monitor*

CYP3A4 Inducers (Strong): May decrease serum concentrations of Clindamycin (Systemic). *Risk C: Monitor*

CYP3A4 Inhibitors (Moderate): May increase serum concentrations of Clindamycin (Systemic). *Risk C: Monitor*

CYP3A4 Inhibitors (Strong): May increase serum concentrations of Clindamycin (Systemic). *Risk C: Monitor*

Fecal Microbiota (Live) (Oral): Antibiotics may decrease therapeutic effects of Fecal Microbiota (Live) (Oral). *Risk X: Avoid*

Fecal Microbiota (Live) (Rectal): Antibiotics may decrease therapeutic effects of Fecal Microbiota (Live) (Rectal). *Risk X: Avoid*

Fusidic Acid (Systemic): May increase serum concentrations of CYP3A4 Substrates (High risk with Inhibitors). Management: Consider avoiding this combination if possible. If required, monitor patients closely for increased adverse effects of the CYP3A4 substrate. *Risk D: Consider Therapy Modification*

Grapefruit Juice: May increase serum concentrations of Clindamycin (Systemic). *Risk C: Monitor*

Immune Checkpoint Inhibitors (Anti-PD-1, -PD-L1, and -CTLA4 Therapies): Antibiotics may decrease therapeutic effects of Immune Checkpoint Inhibitors (Anti-PD-1, -PD-L1, and -CTLA4 Therapies). *Risk C: Monitor*

Kaolin: May decrease absorption of Lincosamide Antibiotics. *Risk C: Monitor*

Lactobacillus and Estriol: Antibiotics may decrease therapeutic effects of Lactobacillus and Estriol. *Risk C: Monitor*

Mecamylamine: Lincosamide Antibiotics may increase neuromuscular-blocking effects of Mecamylamine. *Risk X: Avoid*

Mycophenolate: Antibiotics may decrease active metabolite exposure of Mycophenolate. Specifically, concentrations of mycophenolic acid (MPA) may be reduced. *Risk C: Monitor*

Neuromuscular-Blocking Agents: Lincosamide Antibiotics may increase neuromuscular-blocking effects of Neuromuscular-Blocking Agents. *Risk C: Monitor*

Pectin: May decrease absorption of Lincosamide Antibiotics. *Risk C: Monitor*

Sodium Picosulfate: Antibiotics may decrease therapeutic effects of Sodium Picosulfate. Management: Consider using an alternative product for bowel cleansing prior to a colonoscopy in patients who have recently used or are concurrently using an antibiotic. *Risk D: Consider Therapy Modification*

Typhoid Vaccine: Antibiotics may decrease therapeutic effects of Typhoid Vaccine. Only the live attenuated Ty21a strain is affected. Management: Avoid use of live attenuated typhoid vaccine (Ty21a) in patients being treated with systemic antibacterial agents. Postpone vaccination until 3 days after cessation of antibiotics and avoid starting antibiotics within 3 days of last vaccine dose. *Risk D: Consider Therapy Modification*

Pregnancy Considerations

Clindamycin crosses the placenta and can be detected in the cord blood and fetal tissue (Philipson 1973; Weinstein 1976). Clindamycin injection contains benzyl alcohol, which may also cross the placenta.

Clindamycin pharmacokinetics are not affected by pregnancy (Philipson 1976; Weinstein 1976).

Clindamycin is recommended for use in pregnant patients for the prophylaxis of group B streptococcal disease in newborns (alternative option for patients at high risk for anaphylaxis to penicillin [or whose risk is unknown], and who have GBS susceptible to clindamycin) (ACOG 797 2020); prophylaxis and treatment of *Toxoplasma gondii* encephalitis (alternative therapy),

or treatment of *Pneumocystis* pneumonia (alternative therapy) (HHS [OI adult 2024]); bacterial vaginosis (CDC [Workowski 2021]); or malaria (CDC 2023). Clindamycin is also one of the antibiotics recommended for prophylactic use prior to cesarean delivery and may be used in certain situations prior to vaginal delivery in patients at high risk for endocarditis (ACOG 199 2018).

Breastfeeding Considerations

Clindamycin is present in breast milk.

Clindamycin breast milk concentrations were determined in 5 women immediately postpartum. The dose administered was clindamycin 150 mg by mouth 3 times daily for at least 1 week. Milk was collected over a 6-hour period of time. Maternal serum was collected throughout the study period. Milk concentrations ranged from undetectable (<0.5 mcg/mL) to 3.1 mcg/mL and varied greatly between patients. These concentrations were consistent with the variable maternal serum concentrations of undetectable (<1 mcg/mL) to 33 mcg/mL. The highest breast milk concentrations were observed in the samples from the mothers with the highest serum concentrations (Stéen 1982). Using a milk concentration of 3.1 mcg/mL, the relative infant dose (RID) of clindamycin is 1.2% to 4.7% compared to an infant therapeutic dose of 10 to 40 mg/kg/day, providing an estimated daily infant dose via breast milk of 0.465 mg/kg/day. In general, breastfeeding is considered acceptable when the RID is <10% (Anderson 2016; Ito 2000).

The manufacturer reports that clindamycin breast milk concentrations range from <0.5 to 3.8 mcg/mL following doses of 150 mg orally to 600 mg IV.

One case of bloody stools in an infant occurred after a mother received clindamycin while breastfeeding; however, a causal relationship was not confirmed (Mann 1980). In general, antibiotics that are present in breast milk may cause non-dose-related modification of bowel flora.

According to the manufacturer, the decision to breastfeed during therapy should consider the risk of infant exposure, the benefits of breastfeeding to the infant, and the benefits of treatment to the mother; alternative agents may be preferred. Additional guidelines recommend avoiding clindamycin in breastfeeding patients if possible; monitor breastfeeding infants for GI disturbances, diarrhea, and bloody stools if maternal treatment is required (WHO 2002).

Clindamycin is an alternative antibiotic for the empiric treatment of bacterial mastitis in patients who are breastfeeding. Antibiotic use may be considered when symptoms are present for >24 hours and have not responded to conservative measures, or the patient has symptoms such as fever or tachycardia. Consider a milk culture if symptoms do not

improve after 48 hours of antibiotic therapy. The diagnosis of mastitis does not require interruption of breastfeeding (ABM [Mitchell 2022]; WHO 2000).

Monitoring Parameters

Observe for changes in bowel frequency. Monitor for colitis and resolution of symptoms. In severe liver disease monitor liver function tests periodically; consider monitoring renal function periodically in patients with renal impairment or taking nephrotoxic medications; during prolonged therapy monitor CBC, liver, and renal function tests periodically.

Mechanism of Action

Reversibly binds to 50S ribosomal subunits preventing peptide bond formation thus inhibiting bacterial protein synthesis; bacteriostatic or bactericidal depending on drug concentration, infection site, and organism

Pharmacokinetics (Adult Data Unless Noted)

Absorption: Oral, hydrochloride: Rapid (90%); clindamycin palmitate must be hydrolyzed in the GI tract before it is active.

Distribution: Distributed in body fluids and tissues; no significant levels in CSF, even with inflamed meninges.

Neonates (Gonzalez 2016):

PMA $\leq$ 28 weeks: Median: 1.2 L/kg (range: 0.87 to 2.26 L/kg).

PMA >28 to 32 weeks: Median: 1.3 L/kg (range: 0.74 to 1.88 L/kg).

PMA >32 to 40 weeks: Median: 1.03 L/kg (range: 0.7 to 2.12 L/kg).

Neonates and Infants $\leq$ 5 months (PMA >40 to 60 weeks): Median: 0.99 L/kg (range: 0.64 to 1.27 L/kg) (Gonzalez 2016).

Infants >5 months: Median: 0.83 L/kg (range: 0.7 to 1.17 L/kg) (Gonzalez 2016).

Children $\geq$ 2 years and Adolescents (Smith 2017):

Non-obese: Median range: 0.81 to 0.9 L/kg.

Obese: Median range: 0.86 to 1.03 L/kg.

Protein binding: 94%

Metabolism: Biologically inactive clindamycin phosphate (intravenous formulation) is rapidly converted to active clindamycin. Clindamycin is metabolized predominantly by CYP3A4, with minor contribution by CYP3A5, to form clindamycin sulfoxide (major metabolite) and N-desmethylclindamycin (minor metabolite).

Bioavailability: Oral: ~90%.

Half-life elimination:

Neonates (Gonzalez 2016):

PMA $\leq$ 28 weeks: Median: 5.89 hours (range: 2.42 to 12.9 hours).

PMA >28 to 32 weeks: Median: 5.25 hours (range: 2.34 to 8.87 hours).

PMA >32 to 40 weeks: Median: 3.96 hours (range: 1.3 to 8.83 hours).

Neonates and Infants $\leq$ 5 months (Gonzalez 2016): PMA >40 to 60 weeks: Median: 2.35 hours (range: 0.94 to 6.44 hours).

Infants >5 months to 1 year (Gonzalez 2016): Median: 2.05 hours (range: 1.26 to 3.47 hours).

Children $\geq$ 2 years and Adolescents (Smith 2017):

Non-obese: Median range: 2.15 to 2.84 hours.

Obese: Median range: 2.15 to 3.55 hours.

Adults: 3 hours.

Elderly (oral) ~4 hours (range: 3.4 to 5.1 hours).

Time to peak, serum: Oral: Within 60 minutes; IM: 1 to 3 hours.

Excretion: Urine (~10%) and feces (3.6%) as active drug and metabolites.

Pharmacokinetics: Additional Considerations (Adult Data Unless Noted)

Anti-infective considerations:

Parameters associated with efficacy: AUC₂₄/minimum inhibitory concentration (MIC), goal: $\geq$ ~27 (LaPlante 2008; Levison 2009; Muller 2010).

Expected drug exposure in normal renal function:

C_{max} (peak): Steady state:

Neonates (preterm and term) and Infants <4 months of age: IV: Median dose 15 mg/kg/**day** in divided doses: ~7.9 to 8.9 mg/L (Greenberg 2020).

Children 2 to 6 years of age: IV: 12 mg/kg/dose every 8 hours: 14.1 mg/L (Smith 2017).

Children >6 years and Adolescents: IV: 10 mg/kg/dose every 8 hours: 12.2 mg/L (Smith 2017).

Adults: IV: 600 mg every 6 or 8 hours: 12.2 mg/dL (Flaherty 1988; Smith 2017).

AUC₀₋₂₄ (total): Steady state:

Neonates (preterm and term) and Infants <4 months of age: IV: Median dose 15 mg/kg/**day** in divided doses: 8.3 mg/L: 96 mg•hour/L (Greenberg 2020).

Children 2 to 6 years of age: IV: 12 mg/kg/dose every 8 hours: 132.6 mg•hour/L (Smith 2017).

Children >6 years of age: IV: 10 mg/kg/dose every 8 hours: 134.4 mg•hour/L (Smith 2017).

Adolescents: IV 10 mg/kg/dose every 8 hours: 145.8 mg•hour/L (Smith 2017).

Adults: IV: 600 mg every 6 to 8 hours: 105.6 to 134.1 mg•hour/L (Flaherty 1988; Smith 2017).

Postantibiotic effect: Varies based on the organism and antimicrobial exposure (concentration and duration):

Bacillus anthracis: 2 hours (Athamna 2004).

S. aureus: Mean: 1.7 to 4 hours (range: 0.4 to 6.7 hours) (Xue 1996).

Streptococcus pneumoniae: ~1 to 6.9 hours (Kuenzi 1987).

S. pyogenes: ~1 to 5.5 hours (Kuenzi 1987).

Patient Counseling Points

(For additional information see "[Clindamycin \(systemic\): Patient drug information](#)")

What is this drug used for?

- It is used to treat or prevent bacterial infections.

All drugs may cause side effects. However, many people have no side effects or only have minor side effects. Call your doctor or get medical help if any of these side effects or any other side effects bother you or do not go away:

- Diarrhea, stomach pain, upset stomach, or throwing up
- Bad taste in your mouth

WARNING/CAUTION: Even though it may be rare, some people may have very bad and sometimes deadly side effects when taking a drug. Tell your doctor or get medical help right away if you have any of the following signs or symptoms that may be related to a very bad side effect:

- C diff–associated diarrhea (CDAD) like stomach pain, cramps, or very loose, watery, or bloody stools
- Kidney problems like unable to pass urine, change in how much urine is passed, blood in the urine, or a big weight gain
- Chest pain
- Joint pain or swelling
- Yellow skin or eyes
- Vaginal itching or discharge
- Fever, chills, or sore throat; any unexplained bruising or bleeding; or feeling very tired or weak
- Severe skin reactions; red, swollen, blistered, or peeling skin; red or irritated eyes; sores in the mouth, throat, nose, eyes, genitals, or any areas of skin; fever; chills; body aches; shortness of breath; or swollen glands
- Signs of an allergic reaction, like rash; hives; itching; red, swollen, blistered, or peeling skin with or without fever; wheezing; tightness in the chest or throat; trouble breathing, swallowing, or talking; unusual hoarseness; or swelling of the mouth, face, lips, tongue, or throat

Note: This is not a comprehensive list of all side effects. Talk to your doctor if you have questions.

Consumer Information Use and Disclaimer: This information should not be used to decide whether or not to take this medicine or any other medicine. Only the healthcare provider has the knowledge and training to decide which medicines are right for a specific patient. This information does not endorse any medicine as safe, effective, or approved for treating any patient or health condition. This is only a limited summary of general information about the medicine's uses from the patient education leaflet and is not intended to be comprehensive. This limited summary does NOT include all information available about the possible uses, directions, warnings, precautions, interactions, adverse effects, or risks that may apply to this medicine. This information is not intended to provide medical advice, diagnosis or treatment

and does not replace information you receive from the healthcare provider. For a more detailed summary of information about the risks and benefits of using this medicine, please speak with your healthcare provider and review the entire patient education leaflet.

Brand Names: International

International Brand Names by Country

For country code abbreviations ([show table](#))

(AE) United Arab Emirates: Dalacin c; (AR) Argentina: Clidan | Clindalaf | Clindamicina | Clindamicina duncan | Clindamicina fada | Clindamicina pharmavial | Clindamicina richet | Dalacin c | Dexacne | Microbion | Midilina | Resbiotic | Resbiotic pediatrico; (AT) Austria: Clindac | Clindac Sandoz | Clindamycin | Clindamycin 1a pharma gmbh | Clindamycin lannacher | Clindamycin sandoz | Clindamycin stada | Dalacin c | Lanacine; (AU) Australia: Apo-clindamycin | Calindamin | Chemmart clindamycin | Cleocin | Clindamycin | Clindamycin bnm | Clindamycin link | Clindamycin lu | Clindamyk | Dalacin c | Terry white chemists clindamycin; (BD) Bangladesh: Anobac | Cinamycin | Cleocin | Cleodin | Climed | Climycin | Clinacyn | Clindacap | Clindacin | Clindamet | Clindax | Clinex | Daclin | Dalacin | Dalatic | Endamycin | Fortior | Lincocin | Lindamax | Qcin | Riboclin | Xindal; (BE) Belgium: Clindamycin sandoz | Clindamycine EG | Dalacin; (BF) Burkina Faso: Dalacine; (BG) Bulgaria: Clindamycin | Clindamycin abr | Clindamycin mip | Daclyne | Dalacin c | Klimicin; (BR) Brazil: Clindabiotic | Clindamicil | Clindamin c | Clindarix | Cloridrato de clindamicina | Dalacin c; (CH) Switzerland: Clindamycin mepha | Clindamycin pfizer | Clindamycin sandoz | Clindamycin Zentiva | Dalacin c; (CL) Chile: Biomicin | Clindamicina | Dalacin c; (CN) China: Clindamycin | Dalacin c | De bao sheng | Fu si ka | Hai mai ke | Hu yi | Jing dan | Ke er sheng | Ke lin mei | Lizhu ling | She yue | Shuai jie | Suo ning | Ta di xian | Tang ni wei | Te li xian | Xin fu; (CO) Colombia: Acniben | Baclin | Clinbac | Clindacil | Clindamicina | Dalacin c | Damicine | Damiclin | Euroclin; (CR) Costa Rica: Clindamicina | Clindateg | Dalacin c | Globacin; (CZ) Czech Republic: Dalacin | Dalacin c | Klimicin | Klindamycin; (DE) Germany: Aclinda | Clin sanorania | Clinda 1a pharma | Clinda lich | Clindabeta | Clindadura | Clindahexal | Clindalind | Clindamycin | Clindamycin 1a pharma | Clindamycin abz | Clindamycin Acis | Clindamycin al | Clindamycin aristo | Clindamycin Billix | Clindamycin dura | Clindamycin holsten | Clindamycin klast | Clindamycin ratiopharm | Clindamycin sandoz | Clindamycin von CT | Clindastad | Clindatop | Dalacin | Dentomycin | Sobelin | Turimycin; (DK) Denmark: Dalacin; (DO) Dominican Republic: Cleocin pediatric | Clindamicina | Clindamicina mk | Clindaris | Daltrex | Euroclin | Globacin; (EC) Ecuador: Clindamichem | Clindamicina | Clindamicina mk | Clindana | Cluvax | Dalacin | Dalacin c | Dalacin C Fosfato | Euroclin; (EE) Estonia: Clindamycin | Dalacin c | Klimicin; (EG) Egypt: Clinacyn | Clindacin | Clindacine | Clindam | Clindariv | Clindatech | Dalacin c | Mepaclind; (ES) Spain: Clindamicina qualigen | Dalacin; (ET) Ethiopia: Cindi 150 | Clindamycin abr | Dalacin c; (FI) Finland: Clindamycin navamedic | Clindamycin orion | Clindamycin sandoz | Clindamycin SanoSwiss | Dalacin | Klindagol; (FR) France: Clindamycine mylan | Dalacine; (GB) United Kingdom: Clindamycin | Clindamycin sandoz | Dalacin c; (GH) Ghana: C cin | Dacillin | Entracin | Kinamycin | Klinda 150; (GR) Greece: Dalacin c; (GT) Guatemala: Bactemicina | Dalacin c | Globacin; (HK) Hong Kong: B.b | Climadan | Clindamycin rivopharm | Dalacin c; (HR) Croatia: Dalacin c | Klimicin; (HU) Hungary: Dalacin | Dalacin c | Klimicin; (ID) Indonesia: Albiotin | Anerocid | Biodasin | Cindala | Clidacor | Climadan | Clinatic | Clinbercin | Clindamycin | Clindexin | Clinex | Clinidac | Clinika | Clinium | Clinjos | Clinmas | Comdasin | Dacin | Daclin | Dalacin c | Ethidan | Indanox | Lando | Librodan | Lindacyn | Lindan | Milorin | Niladacin | Nufaclind | Opiclam | Probiotin | Prolic | Xeldac | Zumatic; (IE) Ireland: Clindamycin | Dalacin c; (IL) Israel: Dalacin | Dalacin c; (IN) India: Clid | Clin |

Clincin | Clindaness | Clindanol | Clindaset | Clindasure | Clindatec | Clindatime | Clindol |
 Clingram | Clinivib | Clinsof | Clinviva | Clinzucia | Cliza | Dalacin c | Dalcap | Dalcinex | Delacos |
 Egyclin | Hiclinamycin | Muclin | Nexclin | Xenaclin; (IS) Iceland: Dalacin; (IT) Italy: Dalacin c; (JO)
 Jordan: Clinaram | Clindacin | Clindamyl | Clindox | Clinimycin | Dalacin c | Lanacin; (JP) Japan:
 Dalacin; (KE) Kenya: Bergclin | Cindi 150 | Cindi 300 | Clindacin | Clindar b | Clindazed | Dacibact |
 Dacillin | Dalacin | Dalacin c; (KR) Korea, Republic of: Cleocin | Clicin | Climycin | Clindacin | Dacin |
 Dongkoo clindamycin | Fullgram | Lindaron | Mothers clindamycin | Neotacin | Withus
 clindamycin; (KW) Kuwait: Dalacin c | Dalacin palmitate; (LB) Lebanon: Dalacin | Dalacin c | Danalli |
 Torgyn; (LT) Lithuania: Clindamycin | Dalacin | Dalacin c | Klimicin | Klindamycin; (LU) Luxembourg:
 Clindamycine EG | Dalacin; (LV) Latvia: Clindamycin | Dalacin | Dalacin c | Klimicin | Klindamycin;
 (MA) Morocco: Dalacine; (MX) Mexico: Amonadin | Ancine | Apomiclin | Cigmadil | Clendix |
 Clinamsa | Clindamicin | Clindamicina | Clindamicina avivia | Clindamicina gi se | Clindamicina
 kendr | Clindamicina le gi | Clindamicina Raam | Clindazyn | Cliran | Croci | Dalacin c | Dalacin c
 rtu | Dalafar | Destralina | Drastep | Foshlenn | Indacil | Indamid | Indamid dx | Klamoxyl | Klin
 amsa | Lindacil | Lisiken | Losertrin | Lyrfhis | Midacyn | Noclin | Tanzanita | Ulmicy; (MY)
 Malaysia: Clindac | Dalacin c | Klindam | Lindacin | Tidact; (NG) Nigeria: Clamysol | Dalacin; (NL)
 Netherlands: Clindamycine | Dalacin | Dalacin c; (NO) Norway: Cleocin | Clindamycin | Clindamycin
 navamedic | Clindamycin villerton | Dalacin | Dalacin c | Sobelin; (NZ) New Zealand: Clindamycin
 Abm | Dalacin c; (PA) Panama: Clindacin | Clindalaf | Clindamicina | Clindamicina mk | Dalacin |
 Euroclin | Globacin; (PE) Peru: Clendabrax | Clincheck | Clinda C | Clindabiot | Clindabiotech |
 Clindacin | Clindamax | Clindamed | Clindamicina | Clindapharm | Clindasted | Clindess | Clinlip |
 Clondicyn | Dalacin c | Dalamicina | Dalatribact | Damizit | Euroclin | Inkabiotic | Kilacinox |
 Mecoclin; (PH) Philippines: Abanxl | Acresil | Anerocin | Biogenerics clindamycin | Climaxin | Clin d
 | Clin gen | Clindacin | Clindacyn | Clindagen | Clindagold | Clindal | Clindamycin | Clindamycin
 Pacific | Clindamycin sandoz | Clinmicap | Dacin | Daklin | Dalacin c | Dalamax | Dalamed |
 Dynacin | Indanox | Indimax | Inprosyp-hp | Klindex | Kylezine | May clindamycin | Myclin |
 Peldacyn | Pharex Clindamycin | Potecin | Promeclin | Ritemed Clindamycin | Robecren | Tidact |
 Vagilin | Zindal | Zynamycin; (PK) Pakistan: Clindacure | Clindamycin | Dalacin c | Serind | Uniclin;
 (PL) Poland: Clindacin | Clindamycin | Clindo | Dalacin c | Klimicin; (PR) Puerto Rico: Camyda |
 Cleocin | Cleocin phosphate | Clindamycin | Clindamycin HCL | Clindamycin in 5% dextrose |
 Clindamycin injection in 5% dextrose; (PT) Portugal: Clincina | Clindamicina | Clindamicina atral |
 Clindamicina labesfal | Dalacin c; (PY) Paraguay: Clindamicina dallas | Clindamicina prosalud |
 Cluvax; (QA) Qatar: Cleocin | Cleocin Pediatric | Clindacin | Clinimycin | Dalacin C | Dalacin C
 (IV/IM) | Dalacin C Granules | Klindaver | Lanacin | Sobelin Granules; (RO) Romania: Clindacin |
 Dalacin c; (RU) Russian Federation: Clindacin | Clindafer | Clindamycin | Dalacin c | Klinoksin; (SA)
 Saudi Arabia: Apo-clindamycin | Clindacin | Clindamycin | Dalacin c | Lindacil; (SE) Sweden:
 Clindamycin | Clindamycin actavis | Clindamycin alternova | Clindamycin bijon | Clindamycin
 Orifarm | Clindamycin villerton | Dalacin | Klindamycin ebb; (SG) Singapore: Dalacin c | Tidact; (SI)
 Slovenia: Dalacin | Dalacin c | Klimicin; (SK) Slovakia: Dalacin | Dalacin c | Klimicin; (SR) Suriname:
 Clindamycine; (SV) El Salvador: Camyda | Clindamicina; (TH) Thailand: Acnocin | Clinda | Clinda p |
 Clindacap | Clindaman | Clindarax | Clindastar | Clindin | Clinott | Dacin-f | Dalacin c | Klimicin |
 Klinda rx | Klindamycin | Millda | Rosil | Todacin; (TN) Tunisia: Dalacine; (TR) Turkey: Cleocin | Clin
 Klindan | Klinoksin | Klitopsin | Meneklin; (TW) Taiwan: B.b | Bibi u | Cledomycin | Cleocin | Clicin |
 Clincin | Clindamycin | Clindamycin Patron | Dalcine | Damicin | Kanolin | Kingdacin | Kojarclinda
 | Lindacin | Tidact | Ulecine | Vicin; (UA) Ukraine: Clindahexal | Clindamycin | Dalacin c | Klimicin |
 Ultrex; (UG) Uganda: Cleobact | Clindacin | Dalacin c | Zeemycin; (UY) Uruguay: Clind cap | Clinda |
 Clinda pediatrico | Clindamicina | Clindamicina ion | Dalacin c | Vioclin; (VE) Venezuela, Bolivarian
 Republic of: Clindacin | Clindamicina | Clindox | Daclin | Dalacin; (VN) Viet Nam: Bilclacin |
 Cledamed | Clindamark | Dakina | Fabaclin c | Flamiclinda | Kamicingv | Nakai | Pyclin | Zurer;

(ZA) South Africa: Clindahehexal | Clindamycin actor | Dalacin c; (ZM) Zambia: Clindahehexal | Dalacin c; (ZW) Zimbabwe: Dalacin c

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